Nab-paclitaxel · every 3 weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nab-paclitaxel · every 3 weeks · each treatment day

Active medicines: Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Routine allergy premedicines are usually not required for this albumin-bound formulation. Low-risk antiemetic support is separate; previous reactions may change the plan.
- Nab-paclitaxel is an albumin-bound formulation. It is not interchangeable with ordinary paclitaxel or with the weekly nab-paclitaxel schedule.
- Assess G-CSF according to individual risk and the exact source; it is not automatically prescribed by this template.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00230 v7 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-18 · SACTCheck v0.84.1 · Regimen-level drafts; prescriber and oncology-pharmacy verification required